

Rectourethral Fistula (in Men)

WARWIKI

Patient Information · An abnormal opening between the rectum and the urine tube

A rectourethral fistula is a small tunnel that should not be there. It joins the **rectum** (back passage) and the **urethra** (the tube urine passes through). Urine can leak into the rectum, and gas or stool can get into the urine. This causes repeated infections. It is upsetting, but it can be repaired, usually in planned steps.

About This Condition

Common signs are **bubbles of gas in the urine, urine leaking from the rectum**, stool in the urine, and urine infections that keep coming back. It is not caused by anything you did.

What Causes It

Most often, it is a rare side effect of **prostate cancer treatment**. This includes surgery to remove the prostate, **radiation**, or treatments that freeze or heat the prostate. Less often, it follows rectal surgery, an injury, or Crohn's disease. Fistulas after **radiation** or freezing or heating treatment are harder to heal.

Results in Numbers

nearly
all

close after repair with a muscle flap, if there was no radiation

about
85 in 100

close after repair if there was radiation

about
6 in 10

men who had radiation get their stoma reversed (about 9 in 10 without radiation)

Numbers are averages from published studies. Your own risk may differ; ask your team.

Learn the Terms

Fistula

An abnormal tunnel joining two organs.

Rectourethral

Between the rectum and the urethra.

Pneumaturia

Gas bubbles passing in the urine.

Fecaluria

Stool in the urine.

Diversion

Sending urine, stool or both out another way so the area can heal.

Colostomy

An opening on the belly where stool comes out into a bag.

Gracilis flap

A strip of inner-thigh muscle placed between the repairs to help them heal.

Why might I need a stoma or catheter first? A catheter drains urine away from the fistula. Some men also need a stoma, so stool leaves through the belly. This controls infection and lets the area calm down before the repair. If the fistula came after surgery alone, the stoma is usually reversed later. After radiation or freezing or heating treatment, the stoma is more often permanent.

How It Is Diagnosed

- A look inside the bladder and urethra (**cystoscopy**) and inside the rectum
- **Dye X-rays** and an MRI to map the fistula's size and place
- A review of your past treatment, such as radiation

How It Is Treated

- 1 **Calm things down:** a catheter, and sometimes a stoma, to control symptoms and infection.
- 2 **Repair:** often through the skin between the scrotum and anus. The tunnel is closed in layers.
- 3 **Muscle flap:** healthy muscle, often from the inner thigh, is placed between the repairs, especially after radiation.
- 4 **Reversal:** once healing is confirmed, a stoma may be reversed.

A small fistula with no stool in the urine sometimes closes with a catheter alone. This is rare after radiation. For some men with severe radiation damage, a permanent stoma and a new path for urine is the best choice.

Call the office that is treating you if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Pelvic or rectal pain that is getting worse
- A catheter that leaks or drains poorly

Go to an emergency department if:

- Fever with shaking chills and feeling very ill
- You cannot urinate, or the catheter stops draining
- Heavy bleeding
- You cannot reach the office and are worried

Three Things to Remember

1. This fistula can be repaired. Most men need a few planned steps.
2. Radiation makes healing harder. Ask your team what to expect for you.
3. Some men leak urine after the repair. This can be treated too.